

Cooperative Health Insurance Partnership Discussion

FairBanks x Uganda Health Partners Cooperative (UHPC)

Your health, our mission.

Date	Tuesday, 28 July 2026
Time	3:00 PM - 3:53 PM (EAT)
Platform	Google Meet
Host	Wasswa Wilson (FairBanks)
Purpose	Explore a partnership on cooperative health insurance between FairBanks and UHPC

Attendance

Name	Organisation	Role
Wasswa Wilson	FairBanks	Host
Georginah Byanjeru	UHPC	Lead presenter
Owere David	UHPC	Team member
Johnson Andinda	UHPC	Secretary General
Rebecca Kia	FairBanks	Medical Officer
Diana Kawoozo	FairBanks	Team member
Kigenyi Conrad	FairBanks	Team member (joined after link shared)

1. Opening

Wasswa Wilson opened the meeting. Both organisations had representatives present. The Meet link was set to expire at **4:00 PM**, so the discussion stayed within that window. UHPC was invited to introduce first, then FairBanks, before the main presentation.

2. Introductions

UHPC

- **Georginah Byanjeru** introduced the UHPC team and noted earlier conversations and email (including an expression of interest) had led to this session.
- **Owere David** introduced himself as part of UHPC.
- **Johnson Andinda** introduced himself as Secretary General of UHPC.

FairBanks

- A FairBanks programme colleague welcomed everyone, noted attendance despite a public holiday, and said FairBanks was eager to learn. She was with **Diana**, asked that **Conrad** receive the meeting link, and indicated UHPC would lead the technical explanation.
- **Rebecca Kia** introduced herself as Medical Officer and said she wanted to learn more about the programme.
- **Diana Kawoozo** and **Kigenyi Conrad** were present for FairBanks.

Wasswa then invited UHPC to present.

3. UHPC Presentation - Cooperative Health Insurance

Presenter: Georginah Byanjeru

Who UHPC is

UHPC advances **cooperative health development** so communities can access quality, affordable care and better health outcomes. The vision shared: communities with reliable access to quality, affordable healthcare through health cooperatives.

The problem

Many Ugandan households still face barriers to care:

- High out-of-pocket medical costs (about **41%** cited)
- Delayed care-seeking due to lack of money, leading to complications
- Selling assets or borrowing to pay medical bills
- Financial instability for health providers
- Poor outcomes, including preventable death, when treatment comes late

Core question: How can care stay affordable for families while providers remain financially viable?

How the model works

Cooperative health insurance is a **risk-sharing / pooling** approach:

- 1 Members of an existing group (and often their households) pay a small **prepaid premium** to a preferred healthcare provider.
- 2 When someone needs care, the cost is shared across the pool-not carried by one family alone.
- 3 Members own and govern the scheme (rules, elected board, accountability for spending and performance).
- 4 The **healthcare facility** delivers clinical care, health education, and related services.
- 5 Members may also pay shares, membership, or subscription into the cooperative.
- 6 A **co-pay** at the point of care helps reduce misuse of services.
- 7 Providers may remit a share (about **10%** cited) into a cooperative reserve / solidarity fund.

Benefits for members

- Timely access to quality, affordable care
- Predictable healthcare expenses through regular prepaid payments
- Financial protection (less need to sell assets for medical bills)
- Lower overall health spending through risk sharing
- Health education, prevention, earlier care-seeking, and better household wellbeing

Benefits for providers

- More regular, predictable cash flow for salaries and medical supplies
- Stronger long-term relationships with clients
- Earlier treatment (members already prepaid)
- Better clinical and financial outcomes than unpaid emergency care

Role of UHPC

UHPC builds capacity for health cooperatives and providers by:

- Supporting regulatory compliance and accountability to members
- Helping expand membership and member benefits
- Strengthening cooperative sustainability
- Training for quality, efficiency, and financial sustainability
- Promoting data-driven decisions, research, knowledge sharing, and collaboration

Framing: This is not only paying for treatment-it is investing in healthier families and stronger communities, and supporting Uganda's path toward **universal health coverage**.

Impact figures shared (to about 2024)

Indicator	Figure cited
Health cooperatives registered (Ministry of Trade, Industry and Cooperatives)	About 18
Members with access to care	About 35,000
Healthcare providers offering cooperative health insurance	About 64
Share of health cooperative members who are female	About 57%
Policy support	Regulations and bylaws with the Ministry of Trade to enable expansion

UHPC linked these results to less delayed care-seeking and less selling of assets to pay for care.

4. Questions and Answers

Q1 - Wasswa Wilson (FairBanks)

Impressed by the membership scale, female participation, and facility reach. Asked about special packages, covered services, and sustainability.

UHPC response

- Packages depend on the facility and the **group** that joins.
- Richer cover (for example chronic illness) can be arranged, usually with **higher premiums**.
- Within one joining group, members normally share **one similar package**.
- Covered services depend on what the partner facility can offer; facility and cooperative decide cover based on the agreed premium.
- Sustainability rests on member governance (board), facility participation, regular reviews, clear rules, and UHPC training and oversight-already shown across many facilities.

Q2 - FairBanks (programme / community side)

Before a physical engagement, is there existing literature FairBanks can use to brief stakeholders (including VHTs / community health workers and local leaders), then invite UHPC for a consolidated meeting?

UHPC response

- Materials can be shared; more detail is available online.
- Public materials on cooperative health insurance and related project reports (partners including USAID were mentioned).
- UHPC can also share documents directly.

Q3 - Diana Kawoozo (FairBanks)

Communities have circles, village savings groups, investment clubs, and formal cooperatives. Which groups can UHPC work with?

UHPC response

- Not every informal group is a legal **cooperative**. Registration with the Ministry of Trade, Industry and Cooperatives matters for formal cooperative status.
- Savings and credit groups / SACCOs and similar member groups can still organise for health cover.
- What matters: a group that wants affordable care and can partner with a provider-agree premium, services, and an MoU.
- Workable settings named: community groups, SACCOs/savings groups, schools, employer groups, and similar organised groups.
- UHPC supports communities and **healthcare providers** to set up cooperative health insurance; it is not a conventional commercial insurer selling policies.

5. Partnership Path and Next Steps

Wasswa stressed a real collaboration between **UHPC and FairBanks** for community benefit, and asked whether the next deep-dive should be physical or online.

UHPC clarification

- UHPC does **not** sell insurance as a conventional insurer.
- In partnership, **FairBanks Medical Centre** would be the facility offering cooperative health insurance to members.
- UHPC would **train** FairBanks staff and target group representatives-typically about **15-25 people** over about **three days** on roles, responsibilities, and how the model works.
- FairBanks organises participants; UHPC sends trainers.

FairBanks response

- The training path was accepted.
- Staff training should happen **before** a heavy community rollout so the team understands the programme in detail.
- FairBanks will place this on the **internal meeting agenda for 29 July 2026**, draft an **action plan**, and return to **Georginah and team** with a proposed **roadmap**.

6. Action Points

#	Action	Owner	Timing
1	Share UHPC literature and website materials on cooperative health insurance	UHPC (Georginah & team)	As soon as practical
2	Put UHPC partnership / training on FairBanks internal meeting agenda	Wasswa Wilson / FairBanks	29 July 2026
3	Draft FairBanks action plan and proposed roadmap (deep-dive format and training dates)	FairBanks	After internal meeting
4	Confirm next step with UHPC (training cohort ~15-25 for ~3 days; FairBanks Medical Centre as provider)	FairBanks	After internal alignment
5	Schedule follow-up working session (physical or online) with key stakeholders	FairBanks + UHPC	To be scheduled

7. Closing

Wasswa closed at about **3:53 PM**. He thanked everyone for joining. UHPC offered to keep sharing information as needed. The meeting ended by agreement before the 4:00 PM link expiry.

Agenda Timeline (Summary)

Time	Topic
00:00-02:00	Opening and time limit
02:00-05:40	Introductions (UHPC and FairBanks)
05:40-17:40	UHPC presentation
17:40-23:00	Q&A - packages, cover, sustainability
23:00-32:40	Q&A - literature and eligible groups
32:40-38:20	Partnership path, training, FairBanks follow-up
38:20-39:20	Closing